

NEW IN THE LITERATURE

Single-dose baloxavir for post-exposure prophylaxis of influenza in households

New England Journal of Medicine · published 12 Sep 2026 · PMID [sample]

First seen here today

Household transmission is the setting your influenza pile keeps returning to; this trial reports secondary attack rates with and without prophylaxis.

Matched your topic **influenza household transmission** · checked 20 minutes ago

Acknowledge

Dismiss

Correction: Duration of viral shedding after oseltamivir in hospitalised adults

Clinical Infectious Diseases · published 3 Sep 2026 · PMID [sample]

First seen here 2 Sep 2026

Correction or concern notice — not a retraction

A correction or concern notice is associated with this paper. Review the notice before relying on the paper. This flag does not itself mean the paper was retracted.

Matched your topic **neuraminidase inhibitors** · checked 20 minutes ago

Acknowledge

Dismiss

WORTH A LOOK

Model-generated learning material. Each item shows its own source or evidence support and any hold. Machine review is not clinical validation or human approval. Check current references before applying it.

Antiviral treatment within 48 hours of symptom onset shortens influenza illness, and later treatment still benefits hospitalised patients.

The 48-hour window comes from outpatient trials; observational data in admitted patients support starting antivirals regardless of symptom duration.

Evidence-supported, machine reviewed

Abstract quote matched

Published literature was retrieved and a quote from its abstract was matched to this claim. Not verified, not human-approved, not clinically validated.

Influenza

Antivirals

Inpatient

FROM YOUR SOURCES

IDSA influenza guideline (lecture deck).pptx · slide 14, notes · High

Clinicians should start antiviral treatment as soon as possible for adults with documented or suspected influenza who are hospitalized, irrespective of illness duration.

CHECKED AGAINST

Timing of neuraminidase inhibitor therapy and outcomes in hospitalised influenza

Lancet Respiratory Medicine · 2014 · PMID [sample] · supports

Earlier initiation of neuraminidase inhibitor treatment was associated with reduced mortality, with benefit extending beyond 48 h of symptom onset.

Pre-exposure prophylaxis with oseltamivir is recommended for immunocompromised close contacts.

Conflicting

Held for review

Two retrieved abstracts disagree on the population; this point is shown but Tutor will not ask it.

Influenza

Prophylaxis

FROM YOUR SOURCES

Transplant ID handout 2025.pdf · page 6 · Medium

HELD FOR REVIEW

3 points and 2 questions are held back and are not being shown or asked.

Conflicting evidence between retrieved abstracts

A correction notice was found for a cited paper

Source excluded after the build

Open Sources to see which pile they came from.

Tutor has 41 questions ready and 2 held.

YOUR SOURCES

12 files · 11 readable · 1 needs attention

184,200 of 312,900 extracted-text characters processed; 128,700 remain.

Coverage is of extracted text only, not diagrams, image-only pages or omitted text. Processing does not mean clinical validation.

Source confidence — your judgment of the material's accuracy and usefulness for learning. It is not mastery, priority, difficulty, or evidence that a claim has been verified.

High 2 piles · 5 files

Medium 3 piles · 6 files

Low 1 pile · 1 file

RECENTLY FLAGGED

Had to look up when to add MRSA coverage in severe CAP

Pneumonia

Unsure how long to continue antivirals in a ventilated flu patient

Influenza

Interpretation of a positive Strongyloides serology before steroids

Not filed yet

YOUR LOCAL DATA

An export is readable JSON you can open in any editor. A backup is a consistent copy of the database. Both are written into your data directory on this Mac.

Export as JSON

Back up the database

LITERATURE SETTINGS

Watched topics are short public phrases derived during Build. Edit them freely; only these words are sent to PubMed.

influenza household transmission

neuraminidase inhibitors

influenza vaccine effectiveness

strongyloides screening

Last checked 20 minutes ago · weekly checking is off

Check now

Edit topics

WHERE THIS IS STORED

On this Mac

Your piles, uploaded sources, notes, generated points and questions, answers and flags are saved to a SQLite database and a data directory of your own.

Sent to a model

Build and Grade require an explicit action. Each shows what it will send first. Codex holds your ChatGPT sign-in, so no API key is used.

Sent to PubMed

Short public topic phrases and record identifiers — never source excerpts, filenames or answers.

Kept in this browser

Only what you have typed and not yet saved.

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Only what you have typed and not yet saved.

QUESTION

27 left in this pass · pass 3

In an adult admitted with severe community-acquired pneumonia, which findings should prompt empiric MRSA coverage, and what would let you stop it early?

Evidence-supported, machine reviewed

Abstract quote matched

Published literature was retrieved and a quote from its abstract was matched to the claim, and the question and its reference answer separately passed an assessment against that text. Not verified, not human-approved, not clinically validated.

Pneumonia

MRSA

Antimicrobial stewardship

Your answer

Prior MRSA isolation or colonisation, recent hospitalisation with IV antibiotics, and severe disease (ICU, necrotising or cavitary pneumonia, post-influenza). Start vancomycin or linezolid, send a nasal MRSA PCR and cultures, and stop at 48 h if the PCR is negative and cultures show no MRSA.

No patient identifiers or HIPAA material.

Pressing Grade sends three things, and nothing else:

- the question on screen
- its reference answer and rubric
- the answer you have typed

Where it goes: OpenAI, through Codex on this Mac. No API key is used.

Grade with the model

Show reference answer

Next question

QUESTION

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Evidence-supported, machine reviewed

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● **Partially correct**

You named the main risk factors and a sensible de-escalation trigger. The answer leaves out the strongest single predictor in the cited evidence and overstates what a negative nasal PCR alone can settle.

WHAT YOU DID WELL

Prior MRSA isolation, recent IV antibiotics and post-influenza necrotising pneumonia are the right prompts, and pairing empiric vancomycin or linezolid with a 48-hour reassessment is the stewardship pattern the reference describes.

WHAT IS MISSING OR UNSAFE

Gram-positive cocci in clusters on a good-quality sputum sample is a direct prompt you did not mention. A negative nasal PCR has a high negative predictive value, but the reference stops empiric coverage on the PCR *and* negative cultures at 48 hours, not on the PCR alone.

A BETTER ANSWER

Cover MRSA empirically when there is prior MRSA isolation or colonisation, recent hospitalisation with IV antibiotics, post-influenza or necrotising pneumonia, or gram-positive cocci in clusters on sputum. Send a nasal MRSA PCR and respiratory cultures at the start; stop coverage at 48 hours if the PCR is negative and cultures show no MRSA.

Uncertain: the reference draws on one guideline abstract; thresholds for “recent” hospitalisation vary between sources.

Reference answer

Empiric MRSA coverage in severe CAP is prompted by prior MRSA isolation or colonisation, recent hospitalisation with parenteral antibiotics, post-influenza or necrotising/cavitary pneumonia, or gram-positive cocci in clusters on sputum. Send a nasal MRSA PCR and cultures before starting; a negative PCR with negative cultures at 48 hours supports stopping.

RUBRIC

Names at least three prompting findings (2 pts); names a de-escalation trigger that includes culture results (1 pt); does not recommend routine MRSA coverage for all CAP (1 pt).

WHERE THIS CAME FROM

ATS-IDSA CAP guideline summary.pdf · page 9 ·

High

We suggest not routinely adding MRSA coverage in adults with CAP unless locally validated risk factors are present; obtain cultures and nasal PCR to allow de-escalation.

Next question

QUESTION

27 left in this pass · pass 3

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Where it goes: OpenAI, through Codex on this Mac. No API key is used.

No model grade was recorded. Codex on this Mac could not be reached. Your answer is still here; you can compare it with the reference answer.

Grade with the model

Show reference answer

Next question

Judge it yourself instead

The model did not grade this. If you want the attempt recorded, read the reference answer below and say how you did. It will be stored as **self-assessed**, not as a model grade.

I had it

Partly

I did not have it

Reference answer

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Grade with the model

Show reference answer

Next question

Flag a knowledge gap

Whatever you were unsure about, in your own words. Nothing else is required.

No patient identifiers or HIPAA material.

What were you unsure about?

Had to look up...

Topic (optional)

Saved on this Mac. Not sent anywhere.

Close

Save flag

Neuraminidase inhibitors in hospitalized adults
Clinical Infectious Diseases · published 3 Sep 2026 ·
PMID [sample]

First seen here 2 Sep 2026

Correction or concern notice — not a
retraction

A correction or concern notice is associated with this
paper. Review the notice before relying on the paper.
This flag does not itself mean the paper was
retracted.

Matched your topic **neuraminidase inhibitors** · checked

SOURCES

Add files here to build learning points and Tutor questions. Saving a file or note is local. Open a pile to preview Build learning material, then choose Send this batch to authorize the model checks.

Source confidence — your judgment of the material’s accuracy and usefulness for learning. It is not mastery, priority, difficulty, or evidence that a claim has been verified.

NEW PILE

Antimicrobial stewardship

Low

Medium

High

Create pile

HIGH 2 PILES

Influenza — IDSA guideline and lectures

4 files · 2 notes · 18 points · 14 questions · 71% processed

Add files

PDF, PPTX, DOCX, TXT, MD · text is extracted; scans and image-only slides need a text alternative

FILES IN THIS PILE

PPTX

IDSA influenza guideline (lecture deck).pptx

42 slides · text in 40 of 42 units · 2 image-only · 148,300 chars

High

Exclude

Delete

PDF

Uyeki 2019 IDSA seasonal influenza CPG.pdf

47 pages · text in 47 of 47 units · 96,900 chars

High

Exclude

Delete

MD

Flu grand rounds notes.md

1 unit · 6,200 chars

High

Exclude

Delete

PPTX

Ward teaching — antivirals.pptx

18 slides · text in 3 of 18 units · 15 image-only

Mostly image-only slides. No OCR or diagram interpretation is performed; only 3 slides will be read.

High

Exclude

Delete

BUILD LEARNING MATERIAL

184,200 of 251,400 extracted-text characters processed; 67,200 remain. The next batch continues from where the last one stopped.

Build learning material

Shows exactly which excerpts would be sent before anything leaves this Mac.

PLAIN-TEXT NOTES

Baloxavir PEP household trial — secondary attack rates

Journal club · 2 notes

Use as source

Add a note

Delete this pile

ATS/IDSA community-acquired pneumonia

1 file · 0 notes · 9 points · 7 questions · fully processed

MEDIUM 3 PILES

Transplant ID handouts 2025

3 files · 1 note · 6 points · 4 questions · 40% processed

Board review — nephrology

2 files · 0 notes · 0 points · 0 questions · not started

Journal club, spring

1 file · 4 notes · 2 points · 1 question · fully processed

LOW 1 PILE

Conference slides (unverified)

1 file · 0 notes · 3 points · 0 questions · fully processed

WHERE THE GAPS ARE

Where the gaps are, as a map rather than a list. Drag a topic to move it, tap it to see its flags, pinch or scroll to zoom. Nothing here is a queue; it describes, it does not assign.

Open flags

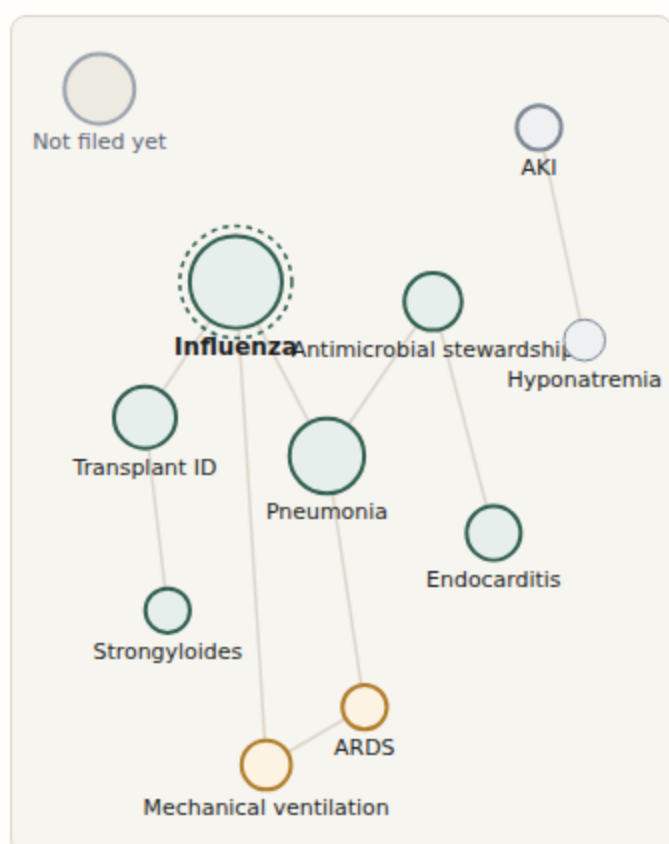
All

By specialty

—

+

Fit



● Infectious disease ● Pulmonary / critical care

● Nephrology ● Not filed yet

· size = open flags · line = shared source or learning point

3 flags have no topic yet. Filing them is the system's job; they are safe where they are.

Influenza

4 open · 2 addressed · linked to Pneumonia, Transplant ID, Mechanical ventilation

Unsure how long to continue antivirals in a ventilated flu patient

flagged 2 days ago

Mark addressed

Baloxavir vs oseltamivir for PEP in immunocompromised contacts

flagged 5 days ago

Mark addressed

When to add empiric MRSA coverage in post-influenza pneumonia

flagged 1 week ago

Mark addressed

Timing of influenza vaccination after rituximab

flagged 2 weeks ago

Mark addressed

Oseltamivir dosing in CRRT

flagged 3 weeks ago

Addressed

Tutor has 14 questions on this topic ready and 1 held.
[Open Sources](#) to see which pile they came from.

MATERIAL BEHIND IT

High 2 piles · 5 files

Medium 3 piles · 6 files

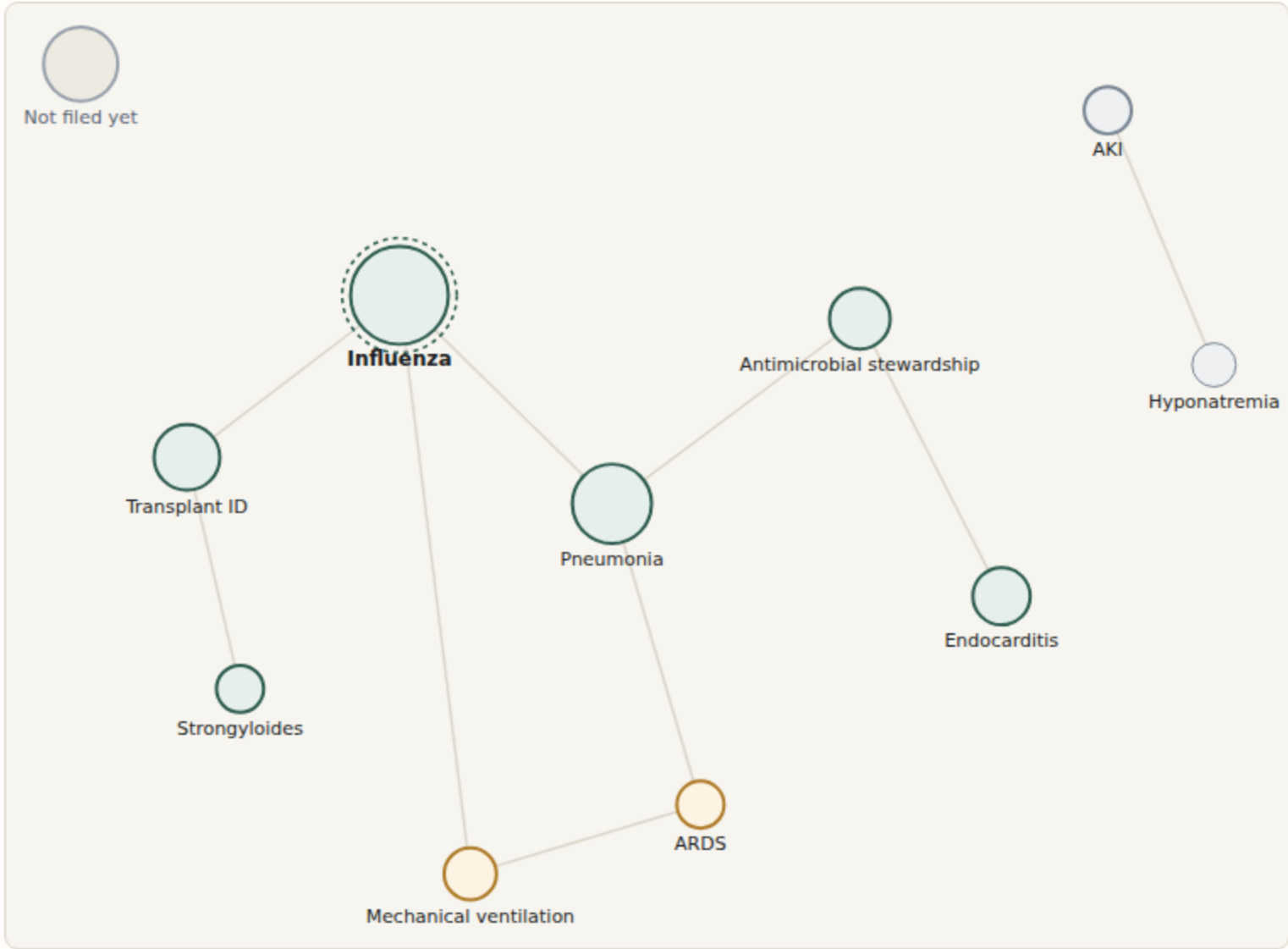
Low 1 pile · 1 file

WHERE THE GAPS ARE

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Open flagsAllBy specialty

−+Fit



● Infectious disease ● Pulmonary / critical care ● Nephrology ● Not filed · size = open flags · line = shared source or learning point

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4 open · 2 addressed · linked to Pneumonia, Transplant ID, Mechanical ventilation

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flagged 2 days ago Mark addressed

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flagged 5 days ago Mark addressed

When to add empiric MRSA coverage in post-influenza pneumonia

flagged 1 week ago Mark addressed

Timing of influenza vaccination after rituximab

flagged 2 weeks ago Mark addressed

Oseltamivir dosing in CRRT

flagged 3 weeks ago Addressed

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MODEL CONNECTION

Signed in

Plus plan

Codex on this Mac is signed in to ChatGPT. Build and Grade will work; nothing has been sent by opening this page.

Check again

Restart the connection

Plan usage

5-hour window

38% used · resets in 2 h 10 min



Weekly

61% used · resets Monday 09:00



Reported by Codex for your ChatGPT plan. Vademecum does not see an account name or address.

WHAT THIS PAGE SENDS

Checking sign-in and usage

Goes through Codex on this Mac, which contacts OpenAI to answer it. No note, question or answer is included.

Build and Grade

Are the only two actions that send study content, and each shows what it will send first. Codex holds your ChatGPT sign-in, and **no API key is used** — model use draws on your ChatGPT plan and its usage limits.

MODEL CONNECTION

Signed out

Codex on this Mac is not signed in to ChatGPT. Reading your notes and questions still works; Build and Grade will not until you sign in.

Check again

Restart the connection

Sign in with ChatGPT

1. Open [the verification page](#) in any browser — this app will not open it for you.
2. Enter this one-time code: **ABCD - EFGH**
3. Come back here and press **Check again**.

The code is shown once and stored nowhere. Closing this page loses it; cancel and start again if that happens.

Check again

Cancel sign-in

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Vademecum — design tokens

Values are the ones in apps/web/src/styles.css; the five tints (accent tint/line, fail bg/line, tag bg) are additions this pass proposes.

COLOR — LIGHT

paper

#f7f5f0

card

#fffefb

line

#dcd7cc

ink

#1c1e22

muted

#5c6470

accent

#2f5d50

accent tint

#e6efec

warn

#7a4a12

warn bg

#fdf3e3

fail

#8a2020

fail bg

#f6e3e3

tag bg

#edeae2

COLOR — DARK

paper

#14161a

card

#1b1e23

line

#2b2f36

ink

#e8e6e1

muted

#9aa2ad

accent

#6fae9b

accent tint

#1d2c28

warn

#e0b877

warn bg

#2a2318

fail

#e08b8b

fail bg

#2c1b1b

tag bg

#23272e

TYPE

- Wordmark · sans 20/600
- Section label · sans 12/600 caps
- Card heading · sans 17/600
- Sub-label · sans 12/600 caps
- Question · serif 20/1.4
- Body · serif 16/1.55
- Small · serif 14/1.45
- Quote · serif italic 15
- Code · mono 20, 0.12em

Vademecum

WORTH A LOOK

Reference answer

WHAT YOU DID WELL

Which findings should prompt empiric MRSA coverage?

Body text is 16px or larger so iOS Safari does not zoom on focus.

Matched your topic · checked 20 minutes ago

Clinicians should start antiviral treatment as soon as possible.

ABCD - EFGH

BADGES — SUPPORT, HOLDS, NOTICES

Evidence-supported, machine reviewed

Draft (source-supported)

Uncertain

Conflicting

Held for review

Correction or concern notice

Retracted

First seen here today

One pill, four tints. Green is reserved for the single strongest claim the product makes; amber for anything held; red only for a retraction.

SOURCE CONFIDENCE AND TOPICS

High

Medium

Low

Filled pills = the owner’s trust in the material, never mastery or evidence.

Influenza

Antivirals

Inpatient

CONTROLS · 44PX TARGETS, 999PX RADIUS

Grade with the model

Show reference answer

Next question

Save flag

Flag a gap

OUTCOME MARKS · 20PX STROKE ICONS

Correct

Partially correct

Incorrect

Self-assessed

SURFACES

CARD

card / line / 12px radius / 16px padding

Pressing Grade sends three things, and nothing else:

the question on screen

its reference answer and rubric

the answer you have typed

Where it goes: OpenAI, through Codex on this Mac. No API key is used.

No patient identifiers or HIPAA material.

Held: warn on warn-bg with a 1px warn border.